residual housing need adjustment within the existing methodology and requested additional information on potential alternatives. Member Lopez also raised questions about the role of high-quality transit areas in the methodology, including how current transit service levels, future transit plans, and input from local transit agencies were considered. She encouraged further discussion regarding how transit accessibility was evaluated and how it might influence future RHNA allocations.

Ms. Johnson explained that the sixth-cycle RHNA methodology relied on the applicable Connect SoCal plan, which incorporated data and forecasting developed through coordination with county transportation commissions and local agencies. She stated that the methodology considered both existing and planned transit investments when evaluating transit accessibility and identifying High-Quality Transit Areas (HQTAs). Ms. Johnson noted that HQTAs were no longer the primary transit planning designation and had since been replaced by other transportation and mobility planning frameworks. She advised that staff would provide a more detailed discussion of transit-related factors and methodologies at a future RHNA Subcommittee meeting focused specifically on transportation considerations.

Regional Council Member Lopez noted that the sixth-cycle methodology relied on projected 2045 transit investments and expressed interest in understanding how those projections compared with current transit capacity and service levels. She suggested that evaluating both planned and existing transit conditions could help inform future RHNA methodology discussions.

Regional Council Member Goodsell stated that his county met many of the characteristics associated with disadvantaged communities and expressed support for Option B as the preferred approach for the seventh-cycle RHNA methodology. He concurred with comments that access to healthcare should be considered as a social equity indicator and noted ongoing local efforts to establish a valley-wide healthcare district. He stated his support for incorporating healthcare access into future methodology discussions.

ADJOURNMENT

Chair Rhodes reported that staff had held their first full consultation meeting with HCD on September 2, which was the first exchange of data and approaches that would inform the RHND. He stated that as had been shared at their first meeting, state law listed the required factors HCD had to consider in its RHND and also outlined that HCD was required to consult with the COG and the California Department of Finance as part of the process. He shared that HCD appeared to have similar data and approaches as SCAG staff had outlined at the August RHNA Subcommittee meeting. He further noted that HCD appeared eager to consider SCAG's recommended approaches. He reported that staff had committed to biweekly meetings until October 15, which was HCD's deadline to issue a determination. He explained that at the first RHNA Subcommittee meeting after